

CLINICAL PRACTICE

Suicide Risk Assessment, Safety Planning & Follow-Up Pack

A structured risk formulation form, a patient-facing safety plan, a lethal-means conversation guide, follow-up protocols, and what to do if the worst happens.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Forms + protocols

VERSION

1.0 · Sept 2026

TIME TO ADAPT

1 hour

WHAT IT IS

A practical set of tools for the highest-stakes part of clinical work: a risk formulation form that shows your reasoning, a patient-facing safety plan, a guide to talking about access to means, a follow-up protocol, and a worksheet for your state's duty-to-protect law.

USE IT WHEN

You are building or tightening your risk-management routine, training a new colleague, or want documentation that will stand up to a chart review.

CUSTOMIZE FIRST

Your state's duty-to-protect statute (Part F), your crisis resources, your after-hours coverage (see the Reciprocal Coverage Agreement), and your firearm and medication counseling scripts.

THIS IS A DOCUMENTATION AND WORKFLOW AID, NOT CLINICAL TRAINING

No tool predicts who will die by suicide. Risk level is a clinical judgment made at a point in time and revisited. If a patient is in imminent danger, call 911 or your local mobile crisis team, and stay with the patient if you can. Use this pack alongside training in a structured approach you know well.

Part A: What a defensible assessment covers

Most claims and board complaints do not turn on whether a clinician saw the future. They turn on whether the clinician asked, thought it through, acted in proportion, and wrote it down. Ask directly and in plain words, and record the patient's answers in their own language where you can.

Domain	What to ask and record
Ideation	Presence, frequency, intensity and duration of thoughts of suicide. Most recent and worst point. Whether thoughts are passive ("wish I would not wake up") or active.
Plan and intent	Whether there is a plan: method, time, place. Whether the patient intends to act, and how strongly. What has stopped them so far.
Preparation	Rehearsal, gathering means, writing notes, giving things away, saying goodbyes, changes in will or insurance.

Domain	What to ask and record
Past behavior	Prior attempts, interrupted or aborted attempts, self-harm, hospitalizations. Ask about the most recent and most serious.
Access to means	Firearms in the home or available, stockpiled or hoarded medication, other methods the patient has considered. See Part D.
Acute changes	Recent loss, humiliation, legal or financial crisis, relationship rupture, discharge from inpatient care, new diagnosis, anniversary dates.
Symptoms that raise risk	Hopelessness, agitation, severe insomnia, panic, psychosis, command hallucinations, intoxication, impulsivity, feeling like a burden or trapped.
Protective factors	Reasons for living, responsibilities, relationships, future plans, faith or values, engagement in treatment, help-seeking, restricted access to means.
Context	Age, identity and discrimination stress, chronic pain or illness, substance use, trauma, isolation, occupation with access to means, family history.
The patient's own view	What they think would help, what has helped before, whom they trust. Their words go in the plan.

TIMES OF HIGHER RISK DESERVE EXTRA STRUCTURE

The days and weeks after discharge from inpatient psychiatric care, after a missed appointment by a patient already at risk, after a major loss, and after a change of clinician or of medication are all times when many clinicians shorten the interval between contacts. Build your protocol around those moments (Part E).

Part B: Risk formulation form (clinician)

Complete at intake for every patient and whenever risk changes. A formulation is a short piece of reasoning, not a checkbox. Include what you considered and decided against.

Patient name or ID

Date and time

Clinician

Setting (office, video, phone) and patient's location

Others present

Element	Findings
Baseline (chronic) risk: history, diagnoses, traits, history of attempts	
Current (acute) risk factors: what is different right now	
Protective factors and reasons for living	
Means: firearms, medication, other, and what was discussed or changed	
Structured measure used and score (for example C-SSRS or ASQ), if any	

Clinical formulation: how the factors fit together, and my reasoning

Risk judgment: low, moderate, high, and why (relative to this patient's baseline)

Confidence and what would change it

What I considered and decided against (for example, hospitalization), and why

Response	Done	Details
Safety plan written or updated with the patient (Part C)		
Lethal-means counseling and agreed changes (Part D)		
Involvement of family or supports (with consent, or as the law allows)		
Consultation with a colleague, supervisor or prescriber		
Change in session frequency or level of care		
Referral or urgent evaluation, transport, or emergency services		
Follow-up contact scheduled (date and method)		
Duty-to-protect analysis, if a third party is at risk (Part F)		

Clinician signature

Date

Part C: Building a safety plan with the patient

A safety plan is a short, written list the patient helps write, in their own words, to use when suicidal thoughts rise. It works best when it is specific, kept where the patient can find it, and reviewed often. The six steps below follow the structure of the Stanley-Brown Safety Planning Intervention. The layout and wording here are original. For the authors' official template, and for training in the full intervention, go to the source (see Sources). The patient handout is the last page of this pack and can be printed or shared on its own.

HOW TO USE IT IN THE ROOM

Write it together, in first person, in the patient's language. Choose specific actions over general ones ("walk to the corner and back" rather than "exercise"). Test for realism: would you actually do this at 2 a.m.? Give the patient a copy, photograph it with their phone if they wish, and offer to share a copy with a trusted person. Revisit it after any crisis and at a regular interval.

Part D: Talking about access to lethal means

Reducing access to the method a person is considering is one of the most practical things a clinician can do. Many suicidal crises are brief, and putting time and distance between a person and the means can be enough for the crisis to pass. Raise it as part of care and as something you do with every patient at risk, not as an accusation.

Sample opening. "When people are having thoughts like these, one thing I ask everyone is about things at home that could be used to hurt yourself, like firearms or medications. Is there anything like that where you live? I ask because I want to help you get through the worst moments with as much safety as possible."

If firearms are present. "What would make it easy for you to keep the firearm somewhere other than your home for now, for example with a family member or friend, a storage facility or a gun shop, or locked and unloaded with ammunition stored separately and the key or code held by someone else?"

Medications. "Would it help to keep only a small supply at home, to have someone else hold the rest, or to use a locked box? I can also adjust how much I prescribe at a time."

Method	Options to discuss
Firearms	Temporary out-of-home storage with a trusted person or a licensed facility or dealer where the law allows. Locked storage with separate ammunition. Someone else holds the key or code. Check your state's laws on transfers and storage before advising.
Medication	Smaller supplies and less frequent refills. Locked box. A family member holds and dispenses. Dispose of unused and expired medication. Include over-the-counter medicines.
Other methods	Ask about the specific method the patient has considered and work out a specific barrier. Rely on the patient's own knowledge of what would be hardest to get around.
Substances	Alcohol and other drugs lower inhibition. Discuss reducing access while at risk.

DOCUMENT WHAT WAS AGREED, NOT JUST WHAT WAS ASKED

Record who holds what, when the change will be made, and the follow-up check. A note that says "discussed means" is much weaker than "patient agreed to give the firearm to his brother by Friday; brother confirmed by phone; will re-check next session."

Part E: Follow-up and continuity protocol

Adopt the protocol below for patients you judge to be at elevated risk, and post it where your covering colleague can see it.

Situation	Protocol (adapt the timing to your practice)
Missed appointment by a higher-risk patient	Contact the patient the same day by phone. Try the emergency contact if you cannot reach them and your consent form allows it. Document each attempt. Escalate per your policy, up to a welfare check.
After a high-risk session	Schedule the next contact for a specific date, sooner than usual. Consider a brief call or message in between.
After emergency care or hospital discharge	Aim for a first outpatient visit within [seven] days of discharge. Ask for the discharge summary and medication list with a signed release. Review and update the safety plan.
Between sessions	Some practices send brief, warm, non-demanding notes or calls to at-risk patients who have dropped out of care. Decide in advance what channels you will use and keep them secure.
When you are away	Give patients a coverage number and confirm your covering colleague has the safety plan and the risk summary.
When care is transferred	Pass on the safety plan and the risk summary with consent and make sure the first appointment with the new clinician happens.

Part F: Duty to protect: your state worksheet

When a patient makes a serious threat against an identifiable person, many states impose or permit a duty to protect, warn or notify. The rules differ greatly: some are mandatory and some permissive, some name specific steps that discharge the duty, some give immunity and some are silent. HIPAA permits disclosure to prevent or lessen a serious and imminent threat to health or safety, but state law decides what you must do. Complete this table once for your state and keep it with your risk protocol.

Question	Answer for [state]
Statute or case law (cite)	
Duty is mandatory, permissive or unclear?	
What triggers it (type of threat, identifiable victim, imminence)?	
Steps that discharge the duty (warn, notify police, hospitalize)?	
Immunity for good-faith disclosure?	
Related rules for firearms (reporting, prohibitions, extreme-risk orders)?	

Question

Answer for [state]

Who to call for advice (carrier risk line, association, attorney)?

Part G: If a patient dies by suicide

The loss of a patient is one of the hardest events in a career, and it is also a legal and professional event. Take these steps in this order and get support.

1. Do not alter, add to or "tidy" the record. If you need to add information, write a dated, labeled late entry, and only after speaking with counsel or your carrier.
2. Notify your professional liability carrier promptly. Many carriers offer a risk-management consultation line at no charge. Ask about counsel before you talk to anyone outside your clinical network.
3. Secure the complete record, including messages, portal messages, voicemail, calendar entries and any psychotherapy notes.
4. Do not discuss the case in writing with anyone but counsel. Respond to legal requests through counsel (see the Subpoena Response Guide).
5. Check your state's rules for what you may share with the family. HIPAA allows limited disclosures to family involved in care and to a personal representative, and state law varies. Many clinicians find that a brief, compassionate condolence, without discussing clinical details, is appropriate and human.
6. Tell your covering colleague, supervisor or consultation group. Ask for support for yourself. Consider a peer who has been through a similar loss (see the Consultation Group Charter).
7. Look after your own wellbeing. Reactions such as guilt, intrusive thoughts and sleep disruption are common. Seek your own care if they persist.

IN PSYALLIANCE

Use the Town Hall specialism channels and your Partner group to compare protocols for after-hours coverage and to organize peer support after a loss. Never post identifying patient information. Set your own state duty-to-protect summary (Part F) as a shared reference in the World library for colleagues in your state.

Handout: My Safety Plan

My name

Date made or updated

Made with

Step 1: My warning signs (thoughts, feelings, situations or behaviors that tell me a crisis may be starting)

1.

2.

3.

Step 2: Things I can do on my own to settle myself

1.

2.

3.

Step 3: People and places that help me feel better or take my mind off things

Name or place

Phone (if a person)

Name or place

Phone (if a person)

Step 4: People I trust and can tell that I am in crisis

Name

Phone

Name

Phone

Step 5: Professionals and services I can contact in a crisis

Who	How to reach them
My clinician: [name]	[office phone] · after hours: [coverage number]
My prescriber: [name]	[phone]
Local urgent care or crisis service: [name]	[phone or address]
988 Suicide & Crisis Lifeline. Emergency: 911	Call or text 988, or chat at 988lifeline.org (Veterans and service members: dial 988, press 1). In an emergency call 911 or go to the nearest emergency department.

Step 6: Making my space safer (what I will do, and by when, to put distance between me and anything I could use to hurt myself)

1.

2.

The one thing that is most important to me and worth living for is:

Sources and further reading

- Stanley B, Brown GK. Safety Planning Intervention: A brief intervention to mitigate suicide risk. Cognitive and Behavioral Practice, 2012. Official template and training: suicidesafetyplan.com.
- Stanley B, et al. Comparison of the Safety Planning Intervention with follow-up vs usual care of suicidal patients treated in the emergency department. JAMA Psychiatry, 2018.
- VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide (health.mil and healthquality.va.gov).
- NIMH Ask Suicide-Screening Questions (ASQ) toolkit (nimh.nih.gov) and the Columbia Suicide Severity Rating Scale (cssrs.columbia.edu).
- 988 Suicide & Crisis Lifeline (988lifeline.org). Suicide Prevention Resource Center (sprc.org). Means Matter, Harvard T.H. Chan School of Public Health (hsph.harvard.edu/means-matter).
- HHS, HIPAA Privacy Rule, 45 CFR 164.512(j): uses and disclosures to avert a serious threat to health or safety (hhs.gov/hipaa).
- APA Ethics Code, Standards 3.04 (avoiding harm), 4.05 (disclosures) and 6.01 (documentation) (apa.org/ethics/code).

ABOUT THIS DOCUMENT

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